A The clinical information-gathering problem

Adult with acute abdominal pain

severity: hidden from agent

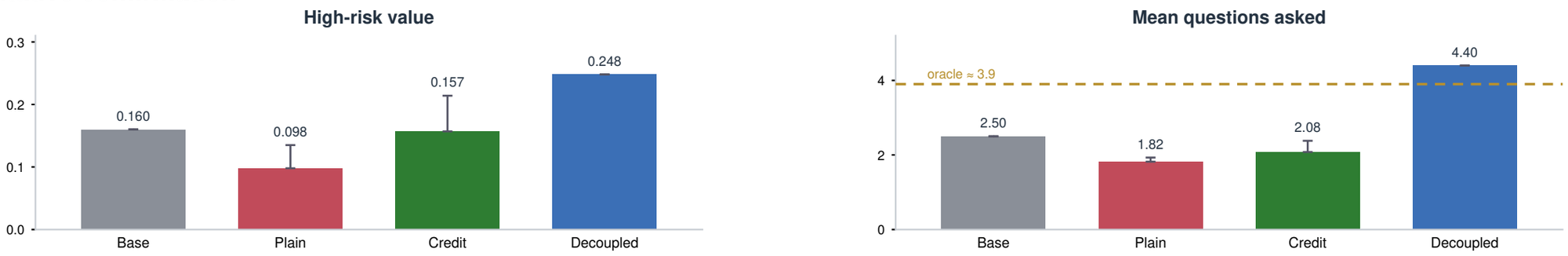
Choose which finding to reveal; each request costs 0.03, then commit when enough evidence is available.

B Same patient, different information-gathering procedures

present · normal

same presentation · same hidden severity · same final verifier · only the training reward differs

C Quantitative confirmation



Held-out diagnostics · pure verifier · no shaping at evaluation. Plain and credit-assignment: mean ± SD across 3 seeds; base and decoupled: single-seed references.

High-risk value = the rare emergent subset — the safety-critical metric.